Docetaxel + pertuzumab + trastuzumab

intravenous (IV) pertuzumab, IV trastuzumab and docetaxel every 21 days. Docetaxel duration is reviewed separately; the two HER2 antibodies may continue after chemotherapy stops while beneficial and tolerated.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart

Trastuzumab / pertuzumab

Keep echo appointments; report breathlessness or swelling.

2 Hands / feet

Docetaxel

Report numbness or tingling before the next dose.

3 Blood / infection

Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

4 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.

5 Fluid / skin / nails

Docetaxel

Report swelling, painful skin or nail changes.

6 Mouth

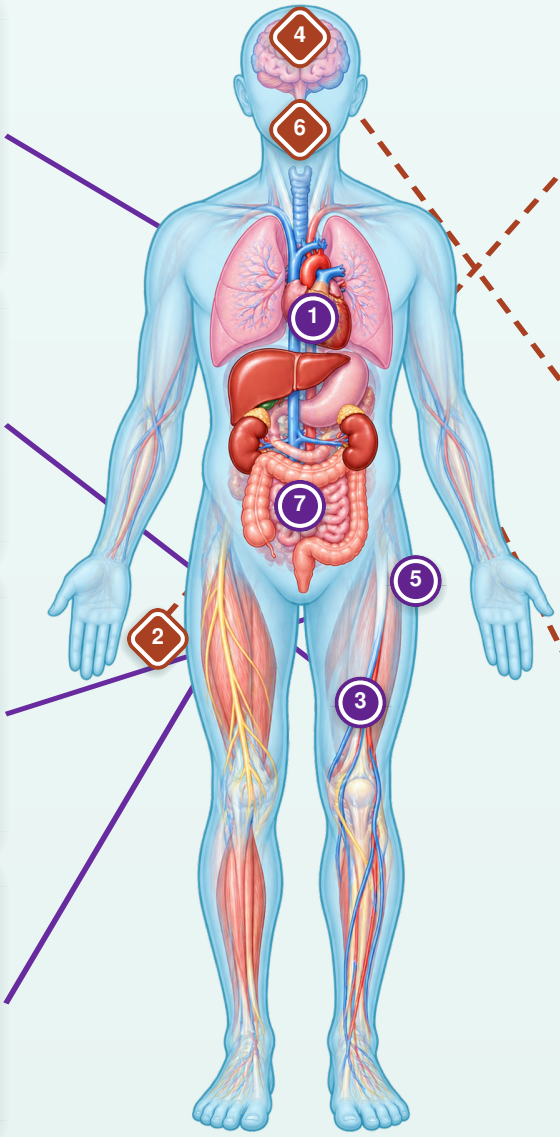
Chemotherapy phase

Painful ulcers or difficulty drinking need advice.

7 Bowel

Pertuzumab / chemotherapy

Report diarrhoea promptly, especially with poor intake.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The map shows selected effects of the medicines named here. Report persistent effects from earlier treatment too.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Docetaxel

- Low blood counts, infection and sepsis
- Hair loss, mouth soreness and fatigue
- Numbness / tingling; sometimes lasting
- Fluid retention and nail changes
- Allergic infusion reaction

Pertuzumab

- Diarrhoea and dehydration
- Infusion / allergic reaction
- Heart pumping-function reduction
- Rash and fatigue

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

The correct supportive prescription

Follow the script for the active medicine, route and treatment day. Do not carry chemotherapy anti-sickness medicines automatically into an antibody-only phase.

Tests before treatment

full blood count (FBC), kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose.

Keep heart checks on the plan

An echo every 12 weeks is separate from blood tests or cancer imaging. Ask who books and reviews it.

Plan for symptoms

Keep your agreed contact numbers and instructions. New symptoms can need assessment before a routine appointment.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00204/

Sources and review status are available online. Follow your own prescription and hospital instructions.